ICSR Clinical Narrative Report

Purpose and scope

This report compiles the supplied source fields into structured Patient, Reaction, Drug and Reporter sections, provides provisional MedDRA PT/SOC mappings, and drafts a case narrative for each case. All records are fictional and intended for educational analysis only, not clinical care or regulatory submission.

Source limitations and audit rules

- Patient initials or an approved patient identifier are absent from the supplied CSV. Age and sex alone do not establish an identifiable patient.
- Only reporter type is supplied (consumer, pharmacist or physician); reporter name/identifier and contact details are absent. Therefore the minimum identifiable patient and reporter criteria are not confirmed.
- Exact calendar dates for treatment start and reaction onset are not supplied; chronology is retained as days after treatment began.
- Clinical findings, treatment for the reaction, hospitalization details, medical history beyond the indication, and other supporting documents are not supplied. Do not infer them.
- Source seriousness labels are transcribed without independently validating the regulatory criteria. 'Medically important' needs documented rationale; a 'Hospitalisation' label needs admission details.
- MedDRA terms/SOCs below are provisional teaching mappings. Confirm exact PT and hierarchy in the licensed MedDRA version required by the assignment. 'Tendon pain' is especially in need of coding confirmation.
- The coding reference gives expectedness status for products/reactions, not MedDRA codes. Expectedness is retained from the source case fields and the synthetic reference version is Synthetic Core Data Sheet v1.0.

Case overview

Case	Age/Sex	Product	Reaction/PT	SOC	Onset	Seriousness	Outcome
SYN-PV-001	30/Male	Amoxicillin	Rash/Rash	Skin and subcutaneous tissue disorders	Day 17	Medically important	Not recovered
SYN-PV-002	36/Unknown	Atorvastatin	Myalgia/Myalgia	Musculoskeletal and connective tissue disorders	Day 1	Not serious	Recovered
SYN-PV-003	34/Female	Metformin	Nausea/Nausea	Gastrointestinal disorders	Day 13	Hospitalisation	Recovered
SYN-PV-004	61/Male	Amlodipine	Peripheral oedema/Peripheral oedema	General disorders and administration site	Day 21	Medically important	Recovered

				conditions			
SYN-PV-005	33/Unknown	Ciprofloxacin	Tendon pain/Pain in tendon	Musculoskeletal and connective tissue disorders	Day 15	Medically important	Not recovered
SYN-PV-006	31/Unknown	Sertraline	Dizziness/Dizziness	Nervous system disorders	Day 3	Medically important	Recovering
SYN-PV-007	63/Male	Omeprazole	Headache/Headache	Nervous system disorders	Day 16	Medically important	Recovered
SYN-PV-008	36/Male	Losartan	Fatigue/Fatigue	General disorders and administration site conditions	Day 2	Not serious	Recovering
SYN-PV-009	78/Unknown	Amoxicillin	Rash/Rash	Skin and subcutaneous tissue disorders	Day 7	Medically important	Recovering
SYN-PV-010	57/Female	Atorvastatin	Myalgia/Myalgia	Musculoskeletal and connective tissue disorders	Day 21	Hospitalisation	Recovered
SYN-PV-011	23/Unknown	Metformin	Nausea/Nausea	Gastrointestinal disorders	Day 19	Medically important	Recovering
SYN-PV-012	24/Female	Amlodipine	Peripheral oedema/Peripheral oedema	General disorders and administration site conditions	Day 4	Medically important	Not recovered

Individual Case Narratives

SYN-PV-001

Patient: Age 30; sex Male. Patient initials/approved identifier were not supplied.

Reaction: Rash. Provisional MedDRA PT: Rash; SOC: Skin and subcutaneous tissue disorders. Provisional; verify against licensed MedDRA version.

Drug: Amoxicillin (20 mg once daily, Oral) for the recorded indication of Anxiety. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 17 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Not recovered at follow-up.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Negative'; rechallenge 'Positive'. These source values have not been independently validated.

Reporter: Type Consumer; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-001: A 30-year-old patient (Male) was reported to have received Amoxicillin (20 mg once daily, Oral) for the recorded indication of Anxiety. Rash was reported 17 day(s) after treatment began. The provisional MedDRA coding is PT 'Rash', SOC 'Skin and subcutaneous tissue disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was not recovered at follow-up. The source states that treatment was stopped. Dechallenge was recorded as 'Negative' and rechallenge as 'Positive'. Concomitant medicines: None reported. Reporter type: Consumer. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-002

Patient: Age 36; sex Unknown. Patient initials/approved identifier were not supplied.

Reaction: Myalgia. Provisional MedDRA PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders. Provisional; verify against licensed MedDRA version.

Drug: Atorvastatin (5 mg once daily, Oral) for the recorded indication of Anxiety. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 1 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovered.

Assessment data: Source seriousness 'Not serious'; expectedness reference 'Known'; dechallenge 'Unknown'; rechallenge 'Negative'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-002: A 36-year-old patient (unknown) was reported to have received Atorvastatin (5 mg once daily, Oral) for the recorded indication of Anxiety. Myalgia was reported 1 day(s) after treatment began. The provisional MedDRA coding is PT 'Myalgia', SOC 'Musculoskeletal and connective tissue disorders'. The source lists the event as 'Not serious' and the expectedness reference as 'Known'. The reported outcome was recovered. The source states that treatment was stopped. Dechallenge was recorded as 'Unknown' and rechallenge as 'Negative'. Concomitant medicines: None reported. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-003

Patient: Age 34; sex Female. Patient initials/approved identifier were not supplied.

Reaction: Nausea. Provisional MedDRA PT: Nausea; SOC: Gastrointestinal disorders. Provisional; verify against licensed MedDRA version.

Drug: Metformin (5 mg once daily, Oral) for the recorded indication of Anxiety. Concomitant medicines: Vitamin D. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 13 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovered.

Assessment data: Source seriousness 'Hospitalisation'; expectedness reference 'Known'; dechallenge 'Negative'; rechallenge 'Negative'. These source values have not been independently validated.

Reporter: Type Consumer; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-003: A 34-year-old patient (Female) was reported to have received Metformin (5 mg once daily, Oral) for the recorded indication of Anxiety. Nausea was reported 13 day(s) after treatment began. The provisional MedDRA coding is PT 'Nausea', SOC 'Gastrointestinal disorders'. The source lists the event as 'Hospitalisation' and the expectedness reference as 'Known'. The reported outcome was recovered. The source states that treatment was stopped. Dechallenge was recorded as 'Negative' and rechallenge as 'Negative'. Concomitant medicines: Vitamin D. Reporter type: Consumer. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-004

Patient: Age 61; sex Male. Patient initials/approved identifier were not supplied.

Reaction: Peripheral oedema. Provisional MedDRA PT: Peripheral oedema; SOC: General disorders and administration site conditions. Provisional; verify against licensed MedDRA version.

Drug: Amlodipine (500 mg twice daily, Oral) for the recorded indication of Anxiety. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 21 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovered.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Positive'; rechallenge 'Negative'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-004: A 61-year-old patient (Male) was reported to have received Amlodipine (500 mg twice daily, Oral) for the recorded indication of Anxiety. Peripheral oedema was reported 21 day(s) after treatment began. The provisional MedDRA coding is PT 'Peripheral oedema', SOC 'General disorders and administration site conditions'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was recovered. The source states that treatment was stopped. Dechallenge was recorded as 'Positive' and rechallenge as 'Negative'. Concomitant medicines: None reported. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-005

Patient: Age 33; sex Unknown. Patient initials/approved identifier were not supplied.

Reaction: Tendon pain. Provisional MedDRA PT: Pain in tendon; SOC: Musculoskeletal and connective tissue disorders. Provisional mapping; confirm exact LLT/PT against licensed MedDRA.

Drug: Ciprofloxacin (500 mg twice daily, Oral) for the recorded indication of Infection. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 15 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Not recovered at follow-up.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Unknown'; rechallenge 'Not done'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-005: A 33-year-old patient (unknown) was reported to have received Ciprofloxacin (500 mg twice daily, Oral) for the recorded indication of Infection. Tendon pain was reported 15 day(s) after treatment began. The provisional MedDRA coding is PT 'Pain in tendon', SOC 'Musculoskeletal and connective tissue disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was not recovered at follow-up. The source states that treatment was stopped. Dechallenge was recorded as 'Unknown' and rechallenge as 'Not done'. Concomitant medicines: None reported. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied

source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-006

Patient: Age 31; sex Unknown. Patient initials/approved identifier were not supplied.

Reaction: Dizziness. Provisional MedDRA PT: Dizziness; SOC: Nervous system disorders. Provisional; verify against licensed MedDRA version.

Drug: Sertraline (20 mg once daily, Oral) for the recorded indication of Hypertension. Concomitant medicines: Paracetamol. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 3 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovering.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Positive'; rechallenge 'Not done'. These source values have not been independently validated.

Reporter: Type Physician; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-006: A 31-year-old patient (unknown) was reported to have received Sertraline (20 mg once daily, Oral) for the recorded indication of Hypertension. Dizziness was reported 3 day(s) after treatment began. The provisional MedDRA coding is PT 'Dizziness', SOC 'Nervous system disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was recovering. The source states that treatment was stopped. Dechallenge was recorded as 'Positive' and rechallenge as 'Not done'. Concomitant medicines: Paracetamol. Reporter type: Physician. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-007

Patient: Age 63; sex Male. Patient initials/approved identifier were not supplied.

Reaction: Headache. Provisional MedDRA PT: Headache; SOC: Nervous system disorders. Provisional; verify against licensed MedDRA version.

Drug: Omeprazole (500 mg twice daily, Oral) for the recorded indication of Hypertension. Concomitant medicines: Paracetamol. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 16 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovered.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Negative'; rechallenge 'Negative'. These source values have not been independently validated.

Reporter: Type Consumer; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-007: A 63-year-old patient (Male) was reported to have received Omeprazole (500 mg twice daily, Oral) for the recorded indication of Hypertension. Headache was reported 16 day(s) after treatment began. The provisional MedDRA coding is PT 'Headache', SOC 'Nervous system disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was recovered. The source states that treatment was stopped. Dechallenge was recorded as 'Negative' and rechallenge as 'Negative'. Concomitant medicines: Paracetamol. Reporter type: Consumer. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-008

Patient: Age 36; sex Male. Patient initials/approved identifier were not supplied.

Reaction: Fatigue. Provisional MedDRA PT: Fatigue; SOC: General disorders and administration site conditions. Provisional; verify against licensed MedDRA version.

Drug: Losartan (5 mg once daily, Oral) for the recorded indication of Infection. Concomitant medicines: Paracetamol. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 2 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovering.

Assessment data: Source seriousness 'Not serious'; expectedness reference 'Unexpected'; dechallenge 'Negative'; rechallenge 'Not done'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-008: A 36-year-old patient (Male) was reported to have received Losartan (5 mg once daily, Oral) for the recorded indication of Infection. Fatigue was reported 2 day(s) after treatment began. The provisional MedDRA coding is PT 'Fatigue', SOC 'General disorders and administration site conditions'. The source lists the event as 'Not serious' and the expectedness reference as 'Unexpected'. The reported outcome was recovering. The source states that treatment was stopped. Dechallenge was recorded as 'Negative' and rechallenge as 'Not done'. Concomitant medicines: Paracetamol. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-009

Patient: Age 78; sex Unknown. Patient initials/approved identifier were not supplied.

Reaction: Rash. Provisional MedDRA PT: Rash; SOC: Skin and subcutaneous tissue disorders. Provisional; verify against licensed MedDRA version.

Drug: Amoxicillin (5 mg once daily, Oral) for the recorded indication of Anxiety. Concomitant medicines: Vitamin D. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 7 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovering.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Positive'; rechallenge 'Not done'. These source values have not been independently validated.

Reporter: Type Physician; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-009: A 78-year-old patient (unknown) was reported to have received Amoxicillin (5 mg once daily, Oral) for the recorded indication of Anxiety. Rash was reported 7 day(s) after treatment began. The provisional MedDRA coding is PT 'Rash', SOC 'Skin and subcutaneous tissue disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was recovering. The source states that treatment was stopped. Dechallenge was recorded as 'Positive' and rechallenge as 'Not done'. Concomitant medicines: Vitamin D. Reporter type: Physician. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-010

Patient: Age 57; sex Female. Patient initials/approved identifier were not supplied.

Reaction: Myalgia. Provisional MedDRA PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders. Provisional; verify against licensed MedDRA version.

Drug: Atorvastatin (5 mg once daily, Oral) for the recorded indication of Infection. Concomitant medicines: Paracetamol. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 21 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovered.

Assessment data: Source seriousness 'Hospitalisation'; expectedness reference 'Known'; dechallenge 'Unknown'; rechallenge 'Negative'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-010: A 57-year-old patient (Female) was reported to have received Atorvastatin (5 mg once daily, Oral) for the recorded indication of Infection. Myalgia was reported 21 day(s) after treatment began. The provisional MedDRA coding is PT 'Myalgia', SOC 'Musculoskeletal and connective

tissue disorders'. The source lists the event as 'Hospitalisation' and the expectedness reference as 'Known'. The reported outcome was recovered. The source states that treatment was stopped. Dechallenge was recorded as 'Unknown' and rechallenge as 'Negative'. Concomitant medicines: Paracetamol. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-011

Patient: Age 23; sex Unknown. Patient initials/approved identifier were not supplied.

Reaction: Nausea. Provisional MedDRA PT: Nausea; SOC: Gastrointestinal disorders. Provisional; verify against licensed MedDRA version.

Drug: Metformin (5 mg once daily, Oral) for the recorded indication of Hypertension. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 19 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Recovering.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Unknown'; rechallenge 'Positive'. These source values have not been independently validated.

Reporter: Type Consumer; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-011: A 23-year-old patient (unknown) was reported to have received Metformin (5 mg once daily, Oral) for the recorded indication of Hypertension. Nausea was reported 19 day(s) after treatment began. The provisional MedDRA coding is PT 'Nausea', SOC 'Gastrointestinal disorders'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was recovering. The source states that treatment was stopped. Dechallenge was recorded as 'Unknown' and rechallenge as 'Positive'. Concomitant medicines: None reported. Reporter type: Consumer. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

SYN-PV-012

Patient: Age 24; sex Female. Patient initials/approved identifier were not supplied.

Reaction: Peripheral oedema. Provisional MedDRA PT: Peripheral oedema; SOC: General disorders and administration site conditions. Provisional; verify against licensed MedDRA version.

Drug: Amlodipine (5 mg once daily, Oral) for the recorded indication of Hypertension. Concomitant medicines: None reported. Treatment was stopped according to the narrative.

Chronology/outcome: Reaction reported 4 day(s) after treatment began; exact calendar dates were not supplied. Outcome: Not recovered at follow-up.

Assessment data: Source seriousness 'Medically important'; expectedness reference 'Known'; dechallenge 'Unknown'; rechallenge 'Positive'. These source values have not been independently validated.

Reporter: Type Pharmacist; reporter name/approved identifier and contact details were not supplied.

Clinical narrative: Case SYN-PV-012: A 24-year-old patient (Female) was reported to have received Amlodipine (5 mg once daily, Oral) for the recorded indication of Hypertension. Peripheral oedema was reported 4 day(s) after treatment began. The provisional MedDRA coding is PT 'Peripheral oedema', SOC 'General disorders and administration site conditions'. The source lists the event as 'Medically important' and the expectedness reference as 'Known'. The reported outcome was not recovered at follow-up. The source states that treatment was stopped. Dechallenge was recorded as 'Unknown' and rechallenge as 'Positive'. Concomitant medicines: None reported. Reporter type: Pharmacist. Patient identifier/initials, exact calendar onset date, reporter name/contact details, and additional clinical findings are not present in the supplied source and must not be inferred. Seriousness and coding require source verification and documented review before any final regulatory use.

Audit/follow-up: Obtain patient identifier and reporter identity/contact details; confirm exact dates, dose, event details, clinical course, and seriousness rationale. Verify MedDRA coding and expectedness against the applicable version/reference before finalization.

MedDRA Coding Summary

Reported term	Provisional PT	SOC	Verification note
Dizziness	Dizziness	Nervous system disorders	Provisional; verify against licensed MedDRA version
Fatigue	Fatigue	General disorders and administration site conditions	Provisional; verify against licensed MedDRA version
Headache	Headache	Nervous system disorders	Provisional; verify against licensed MedDRA version
Myalgia	Myalgia	Musculoskeletal and connective tissue disorders	Provisional; verify against licensed MedDRA version
Nausea	Nausea	Gastrointestinal disorders	Provisional; verify against licensed MedDRA version
Peripheral oedema	Peripheral oedema	General disorders and administration site conditions	Provisional; verify against licensed MedDRA version
Rash	Rash	Skin and subcutaneous tissue disorders	Provisional; verify against licensed MedDRA version
Tendon pain	Pain in tendon	Musculoskeletal and connective tissue disorders	Provisional mapping; confirm exact LLT/PT against licensed MedDRA